

Patient ID: PHolstPeriod: February 22 to March 23, 2024Report Date: March 24, 2024Coverage: 701/734h (86.9%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Feb 22 to Mar 23

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
55	78h	Very High Heart Rate	2	96%

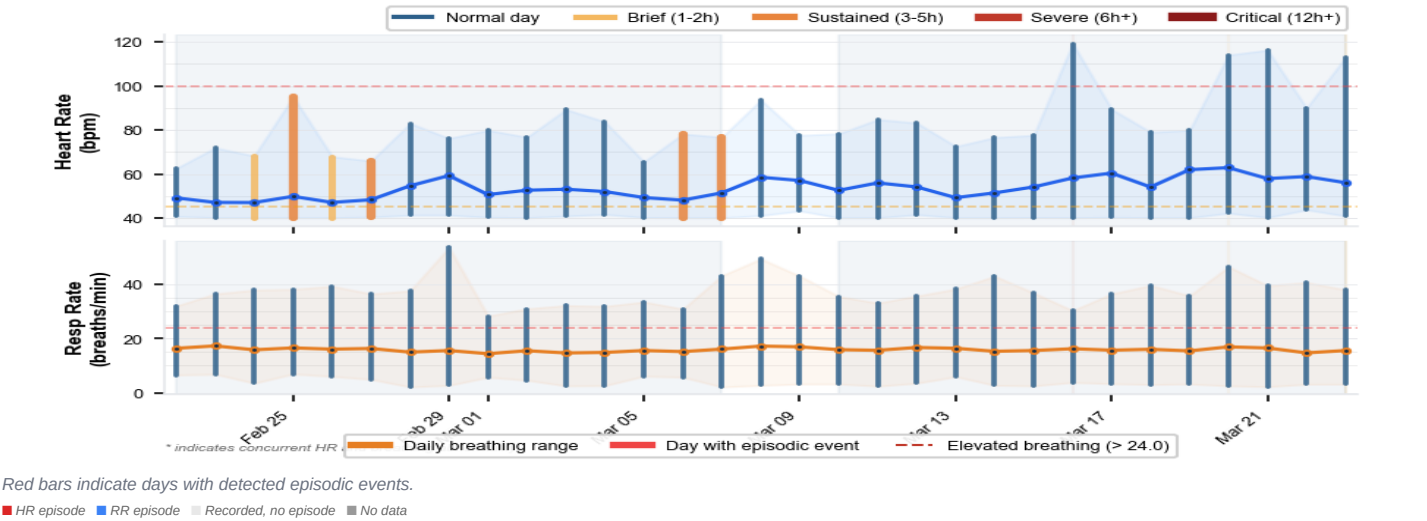
Major Findings: Sustained very high HR (avg 56, peak 119)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Heart Rate	113 bpm	1h	1h	1	56 bpm	Mar 23
2	High Heart Rate	119 bpm	1h	1h	1	58 bpm	Mar 16
3	Low Heart Rate	40 bpm	56h	3h	3	51 bpm	Feb 22 to Mar 07
4	Low Heart Rate	40 bpm	17h	2h	<1	56 bpm	Mar 10 to Mar 23
5	Elevated Heart Rate	107 bpm	1h	1h	1	63 bpm	Mar 20
6	High Breathing	38 brpm	1h	1h	1	16 brpm	Mar 23
7	Elevated Breathing (brief)	39 brpm	1h	1h	1	39 brpm	Mar 09

The P5 to P95 heart rate spread was 26 bpm (43 to 70), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. **High variability:** HR spread 26 bpm (43 to 69).
- 2. **Clustered pattern:** Episodic events on 6 of 31 days (19%).
- 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.



Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

Elevated HR > 100 bpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.